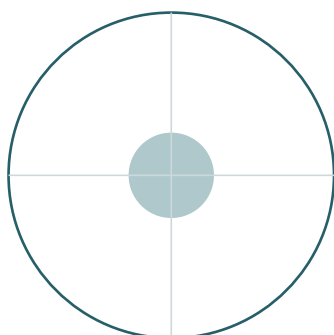


Functional loss can precede a visible disc abnormality

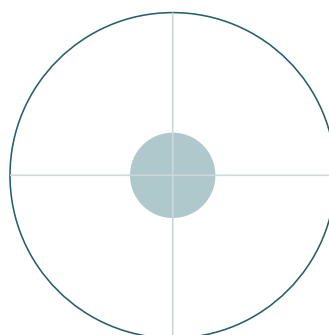
Pain on movement and disproportionate colour loss support optic neuritis even with a normal disc.

A possible optic neuropathy field

Patient view; shaded regions represent impaired vision



Left eye



Right eye

Original pattern examples for left and right eyes. Typical optic neuritis is monocular; actual field defects vary.

Clinical meaning

Typical demyelinating neuritis is subacute and monocular, with pain on movement, colour loss and RAPD. The disc is often normal in retrobulbar disease; papillitis produces swelling. Neuroretinitis instead combines disc inflammation with a later radial macular star. Ask about preceding illness and infectious exposures when that pattern appears.

MRI and recovery answer different questions

MRI assesses demyelination and structural alternatives; ONTT teaching addresses recovery speed.

Treatment and investigation

MRI brain and orbit

Assess white matter lesions and exclude compression.

Natural recovery

Typical demyelinating episodes often improve without treatment.

ONTT result

High-dose IV steroids speed recovery; eventual visual outcome is similar.

Oral-only regimen

Standard-dose oral prednisolone in ONTT increased recurrence.

Clinical meaning

Poor fellow-eye vision or work needs may make faster recovery valuable. Balance that benefit against steroid risks. MRI lesions assess future MS risk, a separate question from current visual recovery. The oral-only finding applies to the specific standard-dose regimen studied, not every oral steroid regimen.